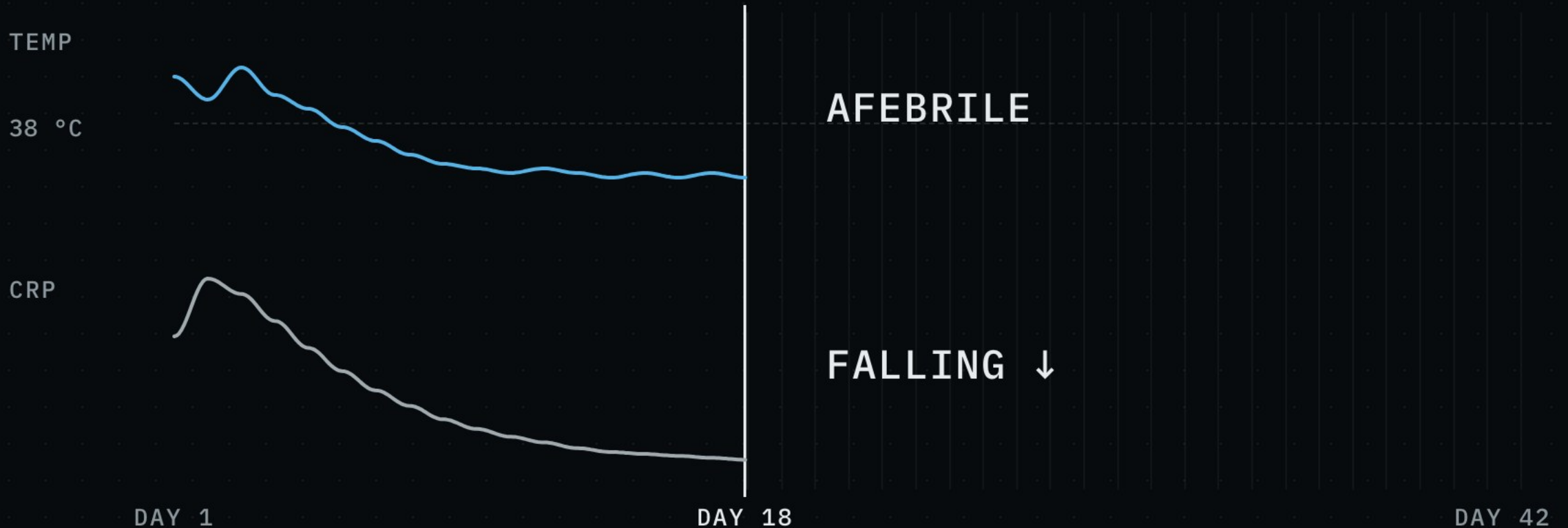


Day 18 of 42. Can we stop?

BED 7 Streptococcal aortic-valve endocarditis

ILLUSTRATIVE PATIENT



Six weeks comes mostly from 1950s observations



Only stable patients were randomized

- Afebrile for 2 days
- CRP < 50% of peak and < 40 mg/L
- WBC < $10 \times 10^9/L$
- Procalcitonin < 0.5 ng/mL
- No progression on TEE
- No further surgery planned

RANDOMIZED

508 stable patients: stop now, or continue

508

Streptococci 57%

S. aureus 24%

E. faecalis 19%

255

TAILORED · STOP NOW

253

STANDARD · CONTINUE TO 4-6 WEEKS

Open-label

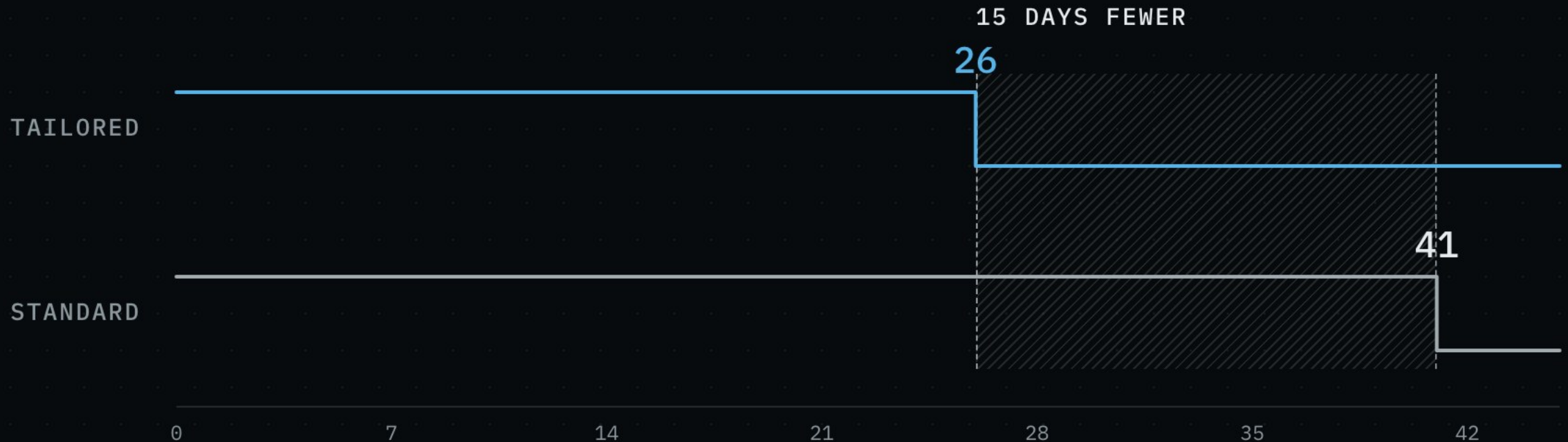
13 centres

Mean age 70

25% women

Tailored therapy cut antibiotics by 15 days

–36.6%



Medians; 36.6% calculated. Illustrative calendar.

13 more days alive without antibiotics

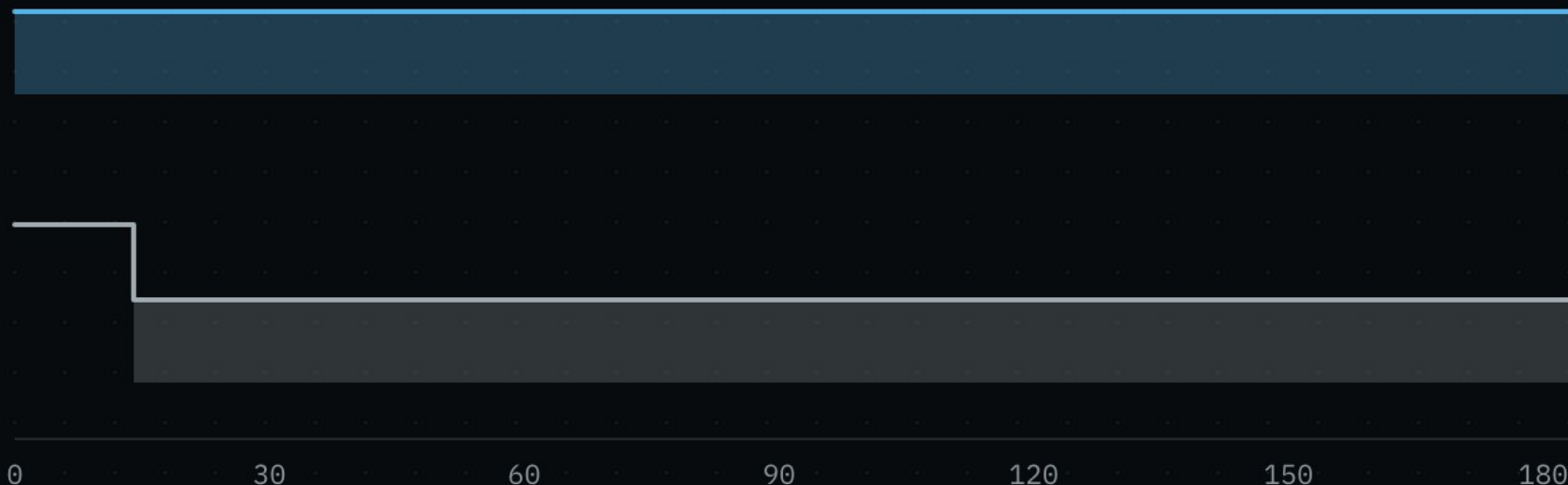
TAILORED

183

STANDARD

169

WITHIN SIX MONTHS



95% CI 12-13 · P < 0.001

MEDIAN

Safety: noninferior at six months

TAILORED



STANDARD



✓ NONINFERIORITY MET

DIFFERENCE



Death

1.18 (0.53-2.63)

Unplanned surgery

0.49 (0.17-1.45)

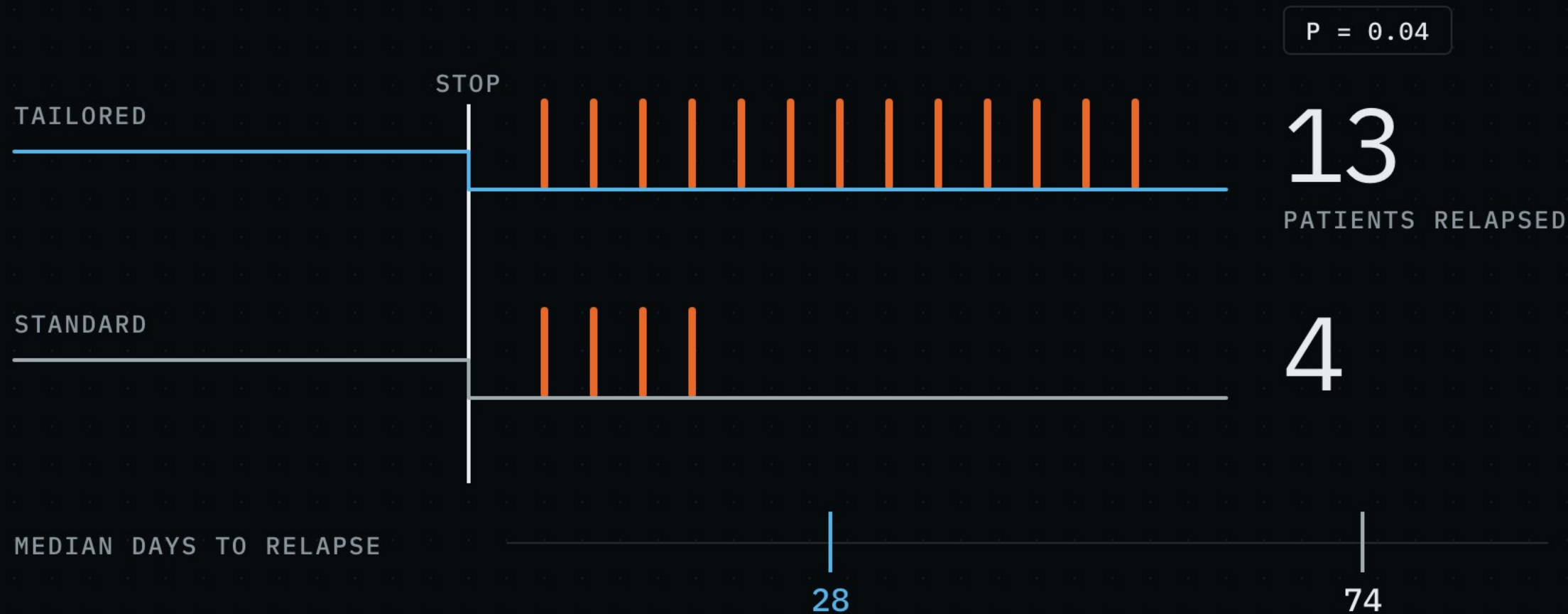
Emboolic events

0.75 (0.26-2.15)

0.25 0.5 1 2
FAVOURS TAILORED FAVOURS STANDARD

Wide intervals: few events

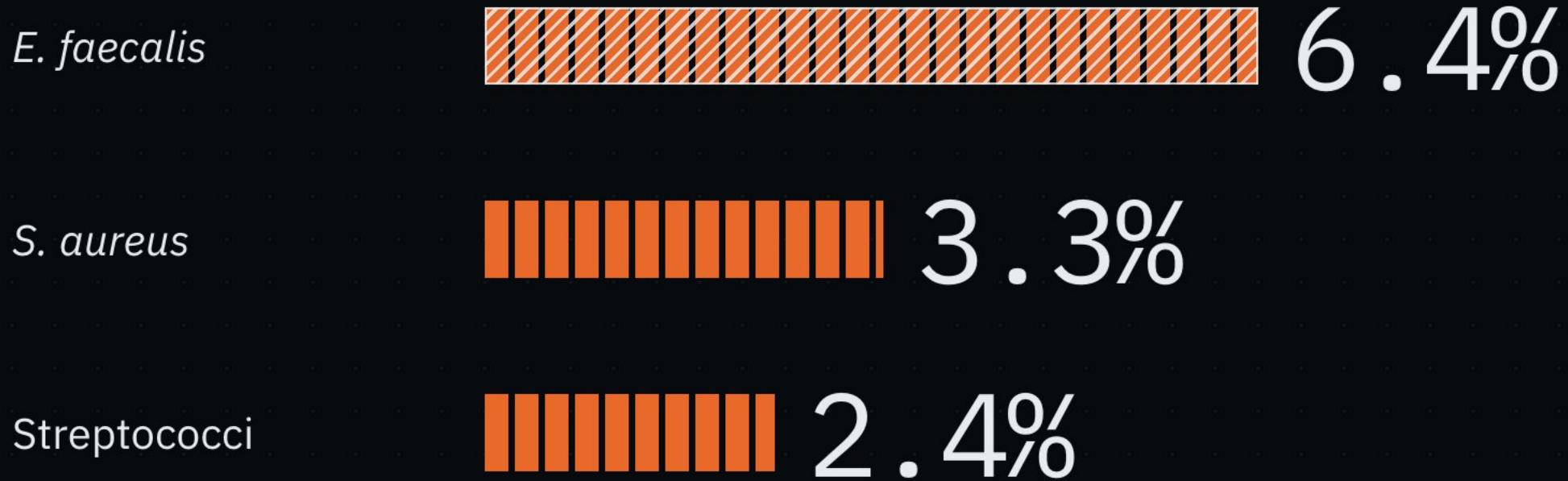
Relapse was more frequent: 5.1% vs 1.6%



Mostly uncomplicated; antibiotics restarted.

E. faecalis relapsed most

RELAPSE BY PATHOGEN



Subgroup: hypothesis-generating

The primary endpoint is partly built in



- Original primary (2020) included relapse
- Real tests: safety, relapse.** (my reading)

Five reasons for caution

- 1 ■ Open-label

- 2 ■ Selected stable patients — about half eligible

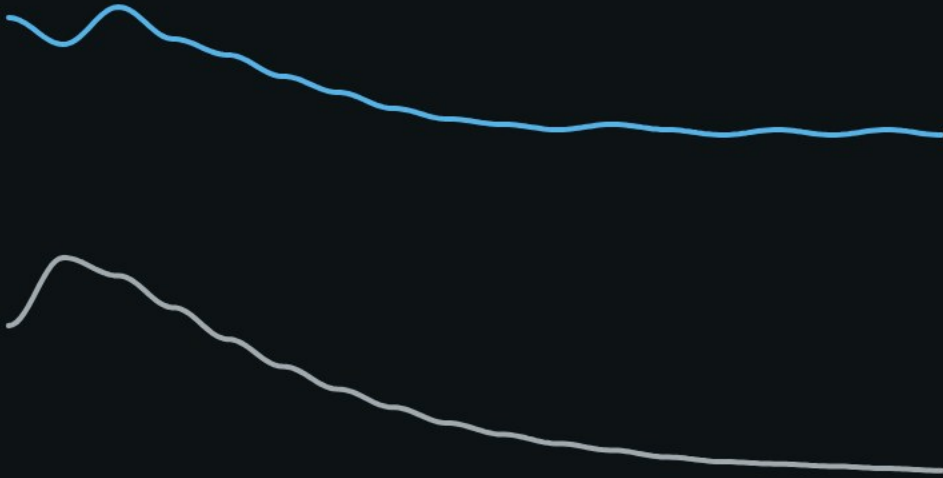
- 3 ■ Specialized centres

- 4 ■ Three pathogens only

- 5 ■ Strict criteria, incl. TEE and procalcitonin

My reading — not a guideline

BED 7



■ All criteria met + strep or *S. aureus*: discuss stopping

▨ *E. faecalis*: be cautious

□ Close follow-up in month one

Would you stop?

BED 7



All six criteria met · streptococcus

STOP

CONTINUE

What would change your mind?

Treat the response, not the calendar.

Questions?



Components of the safety endpoint

SIX MONTHS	TAILORED	STANDARD	HR (95% CI)
Death	5.1%	4.3%	1.18 (0.53–2.63)
Unplanned cardiac surgery	2%	4%	0.49 (0.17–1.45)
Symptomatic embolic events	2.4%	3.2%	0.75 (0.26–2.15)
Composite (primary safety)	21 (8.2%)	27 (10.7%)	Noninferiority met

Absolute difference –2.4 percentage points (95% CI –7.7 to 2.7); noninferiority margin 7.5 percentage points.

Composite, difference and margin: NEJM abstract. Components as reported by TCTMD, 28 Aug 2026.

Stabilization criteria — all required

- Temperature < 38 °C (100.4 °F) for at least 2 days
- CRP < 50% of peak and < 40 mg/L
- WBC < $10 \times 10^9/L$
- Procalcitonin < 0.5 ng/mL
- No progression on TEE within 48 h
- No further cardiac surgery planned
- Before randomization: at least 2–4 weeks of antibiotics

2–4 weeks before randomization: NEJM abstract. Criteria as reported by TCTMD and ACC — full definitions in the NEJM supplement.

Pathogen subgroups

PATHOGEN	SHARE OF TRIAL	RELAPSE	DIRECTION
Streptococci	57.3%	2.4%	Favoured tailored
<i>S. aureus</i>	24.2%	3.3%	Favoured tailored
<i>E. faecalis</i>	18.5%	6.4%	Favoured standard

Consistent across native vs prosthetic valves and medical vs surgical treatment. Adding relapse to the safety endpoint, or excluding extracardiac infections: unchanged.

Subgroups are hypothesis-generating. As reported by TCTMD.

Registered vs published outcome

DESIGN PAPER 2020 · REGISTRY 2021		PUBLISHED 2026
Primary	One safety composite: death, unplanned surgery, embolism, relapse of bacteremia	Efficacy: days alive without antibiotics + safety: death, unplanned surgery, embolism
Relapse	Inside the primary composite	Key secondary: 5.1% vs 1.6%
Planned size	750 (registry, Dec 2021)	508 randomized

The change happened while the open-label trial was recruiting; when and why is not stated in the sources I could read. Authors report that adding relapse back to the safety endpoint left the result unchanged (TCTMD).

Østergaard L, et al. Am Heart J 2020;227:40–46 (PMID 32673830) · ClinicalTrials.gov NCT05144399 (duplicate record, 3 Dec 2021) · NCT03851575 (current).

ESC 2023 guideline – durations

- Streptococci, native valve: 4 weeks (2 weeks with gentamicin in selected uncomplicated cases)
- Streptococci, prosthetic valve: 6 weeks
- *S. aureus* (MSSA): native valve 4–6 weeks; prosthetic at least 6 weeks
- *E. faecalis*: amoxicillin + ceftriaxone, 6 weeks
- Oral step-down after at least 10 days IV in stable patients: class IIa

Delgado V, et al. Eur Heart J 2023;44:3948–4042 (PMID 37622656). Read from an early-online copy – confirm wording in the final version.

POET I — the route question

- Iversen K, et al. NEJM 2019;380:415–424 · PMID 30152252
- Stable left-sided endocarditis
- Partial oral step-down was noninferior to continued IV
- POET I changed the route. POET-II changes the duration.

Background as cited in the ESC Congress 2026 press release.

Who is eligible?

- About half of left-sided IE patients could be eligible (investigators' estimate)
- The three pathogens cause about 70% of IE overall (per authors)
- ESC discussant G. Habib: personalized duration — in selected patients, in specialized endocarditis centres
- He asked for more detail on stabilization criteria, and caution in *E. faecalis*

ESC press release; TCTMD, 28 Aug 2026.

Funding and registration

- Investigator-initiated; funded by Sygeforsikringen “danmark” and others
- ClinicalTrials.gov NCT03851575
- DOI 10.1056/NEJMoa2607887 · PMID 42663314
- ESC Congress 2026, Hot Line 1, Munich, 28 Aug 2026
- PI: Prof. Henning Bundgaard, Rigshospitalet, Copenhagen

Bundgaard H, et al. N Engl J Med, published online 28 Aug 2026 (abstract, PubMed).